

How to Open a Group Home in New Jersey

A step-by-step guide to opening a licensed community residence for individuals with developmental disabilities — the N.J.A.C. 10:44A license, becoming a DDD-approved provider, the twenty-four-month experience requirement, staffing and background checks, admitting individuals, and getting paid

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The New Jersey model — read this first

New Jersey licenses group homes as community residences for individuals with developmental disabilities. The license comes from the New Jersey Department of Human Services (DHS), Office of Licensing (OOL), under N.J.A.C. 10:44A, the Standards for Community Residences for Individuals with Developmental Disabilities, and you operate as an approved provider of the Division of Developmental Disabilities (DDD). A community residence houses up to sixteen individuals and provides food, shelter, personal guidance, and training — it is not a health care facility. Two things shape everything else about opening one: your agency must show at least twenty-four months of direct-care experience with the developmental-disabilities population before you can be licensed for a residence, and you must be an approved DDD provider working with the Division, not an investor opening a home on your own. This guide walks you through the experience requirement, becoming a provider, the home and its staffing, screening and training your team, admitting and supporting individuals, and how a group home is funded.

How to Use This Guide

This guide takes you from idea to an operating New Jersey group home. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective New Jersey providers — not legal, tax, or licensing advice. Requirements, fees, forms, and contacts change, so verify each step with the agency named before you rely on it. Where this guide gives a figure or a timeframe, treat it as a starting point to confirm with the Division, not a final answer.

The one-paragraph version

Build the required track record first: New Jersey will not license you for a residence until your agency has at least twenty-four months of direct-care experience with individuals with developmental disabilities, so most new providers start by delivering non-residential services (day habilitation, community-based supports, respite, or individual supports) through the Community Care or Supports Programs. Become an approved provider by submitting the Combined Application to Become a Medicaid/DDD Approved Provider. Register your business, secure a home, and prepare your policy and procedure manual to DDD standards with your assigned Quality Assurance Specialist. Complete the programmatic audit, attend orientation with the Office of Licensing and DDD, and present your business plan. Screen every staff member — criminal-history background check, Child Abuse Record Information (CARI) check, Central Registry, and drug testing under Stephen Komninos' Law — and complete orientation including the Division's life-threatening-emergency curriculum. Pass the licensing inspection, and only then have individuals move in. A group home is funded through DDD — the Community Care Program waiver and the Supports Program — with individuals contributing to the cost of their room and board.

The Path at a Glance

Ten steps take you from idea to an operating New Jersey group home:

1. **Build the experience.** Accumulate at least twenty-four months of direct-care experience with the I/DD population — usually by starting with non-residential services first.
2. **Form the business.** Register the entity with the New Jersey Division of Revenue; get an EIN; open a business bank account; secure appropriate insurance.
3. **Become a DDD-approved provider.** Submit the Combined Application to Become a Medicaid/DDD Approved Provider and complete the attestation and enrollment.
4. **Secure and plan the home.** Identify a suitable residence and confirm local zoning; plan the staff coverage you will document in the Annex A.
5. **Write your policies.** Develop the written procedure manual and continuous quality improvement system required at N.J.A.C. 10:44A-2.2, with your DDD Quality Assurance Specialist.
6. **Complete the programmatic audit.** Demonstrate that your agency meets DDD quality and compliance expectations.
7. **Attend orientation with OOL & DDD.** Present your business plan and work with the Office of Licensing to finalize residential policies.
8. **Screen and train staff.** Criminal-history, CARL, and Central Registry checks and drug testing; orientation including the Division's life-threatening-emergency curriculum.
9. **Pass the licensing inspection.** Get your N.J.A.C. 10:44A license from the Office of Licensing — before any individual moves in.
10. **Admit individuals and operate.** Accept placements through the Division, support each individual on their Individualized Service Plan, and operate to the rule.

Part 1 — What This Business Is

A group home in New Jersey is a community residence where a small number of adults with intellectual or developmental disabilities live together and receive the supervision, guidance, training, and personal assistance they need — help with daily living, health, medication, money, community activities, and building independence — in a home-like setting that emphasizes normalization, age-appropriateness, least restriction, and person-centered planning. It is licensed by the Department of Human Services, Office of Licensing, under N.J.A.C. 10:44A, and funded through the Division of Developmental Disabilities. This is the model this guide is built around.

Demand is real and durable. New Jersey has thousands of adults with developmental disabilities waiting for residential placements, and the state has invested heavily in expanding community-based options over institutional ones. A well-run group home fills a genuine need — but the flip side is real regulatory structure: an experienced agency, screened and trained staff, a written procedure manual, documented staff coverage, and an Office of Licensing inspection. Meeting those standards is exactly what lets the Division, guardians, and families trust you with a vulnerable person's home and daily care.

It helps to be clear-eyed about what kind of business this is. A group home is an operations and staffing business first: you are responsible for a home, around-the-clock coverage, and the health, safety, and rights of the people who live there. It is not a business you can open cold as an investor — New Jersey deliberately requires you to prove experience first. That makes it slower to enter than in-home care, but it builds a stable, contract-funded service once you are an established DDD provider.

What a community residence is — and is not

- **Group home (community residence):** a provider-owned or leased home for up to sixteen individuals with developmental disabilities — in practice usually far fewer — with staff providing supervision and support; licensed under N.J.A.C. 10:44A. This is the classic group home and the focus of this guide.
- **Supervised apartment:** a more independent community-residence setting, also licensed under N.J.A.C. 10:44A, for individuals who need less intensive support.
- **Community Care Residence (CCR):** a family-caregiver model — an individual living in a caregiver's own home — licensed separately under N.J.A.C. 10:44B. Not covered by this guide.
- **Children's group home:** residential settings for children are licensed by the Department of Children and Families, a different regulator and rule set. Not covered by this guide.

Part 2 — The License: N.J.A.C. 10:44A & DDD Provider Approval

Two approvals stand behind a group home, and it is worth understanding how they fit together. First, your agency must be an approved provider of the Division of Developmental Disabilities — the Division funds and oversees the services. Second, each residence you operate must be licensed by the Office of Licensing under N.J.A.C. 10:44A, which sets the standards for the home itself, its staff, and the care delivered there.

The chapter is organized into six subchapters, and your whole operation is built to them: general provisions and licensure; organization and administration (including the procedure manual and continuous quality improvement system); advocacy and individual rights; service delivery and habilitation; health and safety, including medication and incident reporting; and fire safety and the physical environment. Your policy and procedure manual mirrors this structure, and so does the licensing inspection.

The procedure manual is required by rule

N.J.A.C. 10:44A-2.2 requires every licensee to develop and implement a written procedure manual, reviewed at least annually, designed on normalization, age-appropriateness, least restriction, and person-centered planning — with each procedure stating its title, purpose, steps, staff responsibilities, and recording requirements — plus a continuous quality improvement system. Your assigned DDD Quality Assurance Specialist helps you develop this manual to DDD standards; it is not optional paperwork but the backbone of your license.

It is worth internalizing early that the Office of Licensing and the Division are partners, not adversaries. The Division assigns you a Quality Assurance Specialist precisely because it wants your manual and your program to succeed, and the Office of Licensing inspects to a published rule rather than to surprise standards. Providers who read N.J.A.C. 10:44A closely, ask the Division questions before they guess, and build their operation to the rule from the start have a far smoother path than those who treat licensing as a hurdle to clear once and forget.

Part 3 — The Twenty-Four-Month Experience Requirement

This is the single requirement that most often surprises new entrants, so plan around it from the start. New Jersey requires an agency to show at least twenty-four months of direct-care experience serving individuals with developmental disabilities before it can be licensed to operate a residence. This requirement is not waivable.

The practical consequence is that you almost never open a group home as your first activity. Instead, most new providers begin by delivering non-residential services — day habilitation, community-based supports, individual supports, or respite — through the Community Care or Supports Programs. Doing so builds the experience the state requires, establishes your reputation and relationships with the Division and Support Coordination agencies, and creates a revenue stream that can later support the residential side. Treat the two years not as a delay but as the apprenticeship that makes your first home succeed.

- **Start non-residential.** Offer day, community, individual-support, or respite services to accumulate documented direct-care experience.
- **Document everything.** Keep clear records of the services you provide and the individuals you serve — this is the evidence of your experience.
- **Build relationships.** Work with Support Coordination agencies and the Division; residential placements come through the Division, not open-market marketing.

Part 4 — Becoming a DDD-Approved Provider

To serve individuals and be funded, your agency must be an approved Medicaid/DDD provider. You apply by completing the Combined Application to Become a Medicaid/DDD Approved Provider and the related attestation, and by enrolling as a New Jersey Medicaid provider.

Give the provider-approval process the same seriousness as the license itself — it establishes your agency's standing with the Division, your Medicaid enrollment, and the programs you are approved to deliver. Gaps or inconsistencies here slow everything downstream, so make sure your entity, ownership, and service information are complete and consistent before you submit.

- **Combined Application.** Complete the Combined Application to Become a Medicaid/DDD Approved Provider and the attestation for the services you will provide.
- **Medicaid enrollment.** Enroll as a New Jersey Medicaid provider so DDD services you deliver can be funded.
- **Programmatic audit.** Before residential licensure, expect a programmatic audit confirming your agency meets DDD quality and compliance expectations.
- **Orientation and business plan.** Attend orientation with the Office of Licensing and DDD and present your business plan for the residential program.

Your business plan is where you show the Division you understand the operation you are proposing: who you will serve, how you will staff and supervise the home, how you will keep it financially stable, and how you will meet the rule. A credible plan reflects the real economics of around-the-clock staffing against DDD funding, not optimistic assumptions, and it is the document that turns your experience and your provider approval into a concrete, fundable home.

Part 5 — The Home & the Annex A

The residence itself must be a safe, home-like place that meets the physical-environment and fire-safety standards of N.J.A.C. 10:44A — working smoke and carbon-monoxide detection, a fire-alarm system and extinguishers, clear exits, safe utilities, adequate space and privacy, and the ability to evacuate the home promptly (with a three-minute benchmark). Because you serve people who may need help to evacuate, fire safety and staffing are planned together.

Location matters in two practical ways. First, confirm local zoning early; community residences have legal protections, but you still need to clear zoning and any local approvals for the property. Second, plan the staff coverage the home will need. That coverage is documented in the Annex A — the staff-coverage document the Office of Licensing reviews at the licensing inspection and at contract renewal. Staffing the home to the Annex A, around the clock, is one of the most important safety commitments you make.

Staff coverage is a safety commitment

The Annex A is not paperwork you file and forget. It states the coverage each home provides, and the Office of Licensing holds you to it. Any reduction in coverage for an individual assessed as able to be unsupervised for a set time must be supported by the interdisciplinary team's determination and the Division's documented approval — never done quietly to save money.

Part 6 — Staffing, Background Checks & Training

Your individuals are only as safe as your staff, and New Jersey expects you to screen, train, and verify every person before they have contact with the people you serve. This is among the most heavily inspected parts of the rule, and for good reason.

Screening is layered because a single check does not catch everything. New Jersey requires a fingerprint-based criminal-history background check, a Child Abuse Record Information (CARI) check through the Department of Children and Families, screening against the Central Registry of Offenders Against Individuals with Developmental Disabilities, and drug testing under Stephen Komninos' Law. Complete these before a staff member works with individuals, and act promptly on any disqualifying result.

- **Criminal-history background check.** A fingerprint-based check; you may not employ a person convicted of a disqualifying crime, including a crime involving abuse of a person with a developmental disability, except as the Rehabilitated Convicted Offenders Act permits.
- **CARI check.** Assure through the Child Abuse Record Information check that no employee is listed on the Department of Children and Families' Child Abuse Registry.
- **Central Registry.** Do not employ any person placed on the Central Registry of Offenders Against Individuals with Developmental Disabilities; a person placed on it must cease all contact.
- **Drug testing (Stephen Komninos' Law).** Test employees for controlled dangerous substances as required under N.J.S.A. 30:6D-9.5, with written procedures assuring compliance.
- **Orientation and training.** Complete new-employee orientation under N.J.A.C. 10:44A-2.6 — including unusual incident reporting, emergency procedures, an overview of developmental disabilities, and the Division's life-threatening-emergency curriculum — with signed acknowledgment, plus disability-specific training within the first ninety days.

Two laws every staff member must know

Under Danielle's Law (N.J.S.A. 30:6D-5.1 et seq.), staff must call 911 in a life-threatening emergency and report the call to the Department — with civil penalties for failing to. Under Stephen Komninos' Law, any employee or volunteer who fails to report suspected abuse, neglect, or exploitation is a disorderly person and may be fined \$350 for each day the violation goes unreported. Train both from day one.

Part 7 — Individuals: Admission, Rights & the ISP

How you admit and support individuals is the heart of the rules. Get this right and much of the inspection takes care of itself.

Placements come through the Division, not the open market. When an individual is referred, admit them only when your home can genuinely meet their medical, behavioral, and supervision needs within its license and staffing. From there, everything centers on the person: their rights, their Individualized Service Plan, and their record.

- **Admit only who you can serve.** Gather pre-admission data — diagnosis, psychological evaluation as available, and developmental and behavioral history — and confirm the home can meet the individual's needs.
- **The Individualized Service Plan.** Support each individual on their ISP, developed through person-centered planning with the individual, guardian, Support Coordinator, and team; you carry out the residential goals and report changes so the plan stays current.
- **Individual rights.** Honor rights to dignity, privacy, choice, personal funds and possessions, family contact, and grievances without reprisal — and apply the least restriction necessary, only as permitted.

- **The individual record.** Maintain an individual file at the residence (the client record is the property of the Department) with guardianship, pre-admission data, a personal-possessions inventory, authorizations, and identification of capabilities and needs.
- **Protect and report.** Report suspected abuse, neglect, or exploitation immediately to the DDD hotline at 1-800-832-9173 (press 1), report unusual incidents through the Department's system, and call 911 in an emergency.

The through-line of this part is person-centeredness made real. The individuals in your home are adults with their own preferences, relationships, and goals, and the rule expects your home to support them to live as independently and as fully in the community as they can — not simply to be safe and supervised. Inspectors and Support Coordinators notice the difference between a home that manages people and one that supports them, and so, most importantly, do the individuals who live there.

Part 8 — Getting Paid

A group home is funded through the Division of Developmental Disabilities, not private pay in the way a home-care agency is. Understanding the funding is essential to a realistic business plan.

- **The Community Care Program (CCP) waiver.** The Medicaid waiver (N.J.A.C. 10:46C) that funds residential and supportive services for eligible individuals who meet the ICF/IID level of care; the primary funding path for group-home residents.
- **The Supports Program.** Funds services for eligible individuals living in their own or the family home; relevant to the non-residential services you may deliver while building experience.
- **Contribution to care.** Individuals contribute to the cost of their room and board, handled under N.J.A.C. 10:46D and Division guidance; the residency agreement identifies the individual's contribution and payments.
- **Bill only for services provided.** Deliver the coverage documented in your Annex A and bill accurately; program-integrity review is a normal part of being a provider.

Because funding follows the individual and placements come through the Division, your revenue depends on filling your licensed beds with appropriate placements and staffing them well. Model your real costs — around-the-clock staffing, the home, food, insurance, administration, and training — against the funding for the individuals you serve before you commit.

Part 9 — The Licensing Inspection & Your First 90 Days

The Office of Licensing inspects your residence against N.J.A.C. 10:44A before licensing it, and periodically and on complaint thereafter. The inspection looks at your procedure manual and CQI system, personnel files and background checks, staff coverage against the Annex A, individual records, medication practices, incident reporting, rights, and fire and physical-environment safety.

Treat your first ninety days as the period that sets your habits. The routines you build now — how you document medication, how quickly you report incidents and notify guardians, how you run fire drills across shifts, how you keep individual records current — are exactly what future inspections examine. Build them correctly from your first individual and they simply become how the home runs.

- **Have the manual live in practice.** Staff should be able to describe and follow the procedures they carry out, not just point to the binder.
- **Keep records inspection-ready.** Individual files at the residence, personnel files complete with all checks and training, medication records accurate, incident and drill logs current.
- **Report and notify on time.** Report unusual incidents within required timeframes and notify guardians within the required window — two hours for injuries tied to abuse, neglect, or exploitation or any moderate or major injury.

Part 10 — Common Mistakes to Avoid

- **Trying to skip the two years.** The twenty-four-month experience requirement is not waivable; plan to build it through non-residential services first.
- **Treating the manual as paperwork.** A generic or borrowed manual fails; yours must be specific to N.J.A.C. 10:44A and lived by your staff.
- **Running below your Annex A.** Quietly under-staffing a home is where preventable harm and serious findings begin.
- **Delaying incident reports or guardian notice.** The timeframes are firm; under-reporting or delay is itself a serious compliance failure.
- **Hesitating on 911.** Danielle's Law makes calling 911 in a life-threatening emergency a bright-line duty; never delay to call a supervisor first.
- **Weak background screening.** Skipping or rushing the criminal-history, CARI, Central Registry, or drug-testing checks risks both individuals and your license.

Part 11 — Staying Licensed & Growing

Once you are licensed, staying licensed is about consistency: keep the procedure manual current and reviewed annually, run your continuous quality improvement system so incident and quality data actually drive changes, keep every staff member's screening and training up to date, and maintain the staff coverage you committed to. When the Division amends the rule or issues new circulars, update your procedures and re-train.

Growth usually means adding residences one at a time, each separately licensed and staffed, or expanding the non-residential services that feed your experience and relationships. Grow only as fast as you can staff and supervise well — in this field, quality and compliance are the reputation that earns you the next placement.

Part 12 — What It Costs to Start

Costs vary widely with whether you own or lease the home, its size and condition, your staffing model, and your region of New Jersey. Rather than a single number, budget by category and confirm current figures as you plan:

- **The experience runway.** The cost of operating non-residential services for the two years you build experience — often the largest early investment of time and money.

- **The home.** Purchase or lease, any modifications for safety and accessibility, furnishings, and fire and life-safety equipment.
- **Staffing.** Around-the-clock direct-care coverage to the Annex A, plus supervision and administration — the largest ongoing cost.
- **Screening and training.** Background checks, CARI, Central Registry, drug testing, and the required orientation and training for every hire.
- **Business and compliance.** Entity registration, insurance, Medicaid/DDD provider enrollment, policy development, and any consulting help for your first residence.

Two Worked Examples

These sketches show how the path comes together in practice. They are illustrative, not prescriptive — your own timeline, costs, and staffing will differ.

Example 1 — The day-program provider who opens a first group home

An agency has spent the last two and a half years providing day habilitation and community-based supports to adults with developmental disabilities through the Supports and Community Care Programs. It has real relationships with several Support Coordination agencies, a clean compliance record, and documented direct-care experience well past the twenty-four-month requirement. Deciding to add residential services, the owner completes the residential side of DDD provider approval, leases a four-bedroom single-family home in a residential neighborhood, and confirms zoning. Working with an assigned DDD Quality Assurance Specialist, the agency develops its N.J.A.C. 10:44A procedure manual and continuous quality improvement system, and plans the around-the-clock staff coverage it documents in the Annex A. It screens its residential staff — criminal-history, CARI, Central Registry, and drug testing — and completes orientation including the Division's life-threatening-emergency curriculum. After the programmatic audit and orientation with the Office of Licensing, the home passes its licensing inspection, and the Division places its first individuals. Because the agency built its habits during its day-program years, incident reporting, medication documentation, and fire drills are second nature from day one.

Example 2 — The experienced operator expanding carefully

An established New Jersey provider already operates two licensed group homes and wants to add a third to serve individuals coming off the Community Care Program waiting list. The experience requirement is long since met, and provider approval is in place, so the work is concentrated on the new residence: securing and preparing a suitable home, confirming zoning and fire safety, planning and staffing the Annex A coverage for the specific individuals expected, and adapting the agency's existing procedure manual to the new home. The provider is disciplined about not growing faster than it can staff and supervise well — it recruits and trains the new home's team before opening rather than after, and it treats the third home's first ninety days with the same care as its first home's. Each residence remains separately licensed and separately accountable, and the provider's reputation for quality is what earns it the next round of placements from the Division.

Who to Call & What to Ask

For...	Who to contact
Forming your business	New Jersey Division of Revenue and Enterprise Services (nj.gov/treasury/revenue). Ask: how do I register my entity and obtain the registrations I need to operate?
Becoming a DDD provider	NJ Division of Developmental Disabilities — provider enrollment (nj.gov/humanservices/ddd). Ask: how do I complete the Combined Application to Become a Medicaid/DDD Approved Provider, and what services can I be approved for?
Licensing your residence	NJ DHS, Office of Licensing (nj.gov/humanservices/ool). Ask: what does the N.J.A.C. 10:44A licensing process require, what is the programmatic audit, and what are current timeframes?
Your policy manual	Your assigned DDD Quality Assurance Specialist. Ask: how do we develop the procedure manual and CQI system to DDD standards for my residence?
Background checks	NJ DHS / DDD background-check process and DCF CARI Unit (nj.gov/dcf). Ask: how do I complete the criminal-history, CARI, and Central Registry checks and drug testing before staff have contact?
Funding & Medicaid	NJ DDD and the Division of Medical Assistance and Health Services. Ask: how are group-home services funded through the Community Care Program waiver, and how

For...	Who to contact
	does contribution to care work?
Reporting abuse or an incident	DDD Abuse & Neglect Hotline 1-800-832-9173 (press 1), 24/7; report unusual incidents to the DDD Office of Risk Management through the Department's system; 911 for emergencies.
Fire & building safety	Your local fire official and construction/zoning office, with the DHS Office of Licensing. Ask: what fire, life-safety, and zoning approvals does my community residence need?

Key Terms

- **Community residence:** New Jersey's term for a group home or supervised apartment for up to sixteen individuals with developmental disabilities, licensed under N.J.A.C. 10:44A.
- **DHS Office of Licensing (OOL):** the Department of Human Services office that licenses and inspects community residences.
- **DDD:** the Division of Developmental Disabilities — approves providers, funds services, and oversees the system.
- **Annex A:** the staff-coverage document reviewed at licensing inspection and contract renewal.
- **Individualized Service Plan (ISP):** each individual's person-centered plan of supports; the residence carries out its residential goals.
- **Support Coordinator:** the person, from a Support Coordination Agency, who develops and monitors an individual's ISP.
- **CARI:** the Child Abuse Record Information check through the Department of Children and Families.
- **Central Registry:** the Central Registry of Offenders Against Individuals with Developmental Disabilities.
- **Danielle's Law:** requires staff to call 911 in a life-threatening emergency and report the call to the Department.
- **Stephen Komninos' Law:** requires drug testing and mandatory reporting; failing to report carries a \$350-per-day penalty.
- **Community Care Program (CCP):** the Medicaid waiver (N.J.A.C. 10:46C) that funds residential services for eligible individuals.

Core Rules & References

- N.J.A.C. 10:44A — Standards for Community Residences for Individuals with Developmental Disabilities (DHS Office of Licensing / DDD)
- N.J.S.A. 30:11B-1 et seq.; N.J.S.A. 30:6-32 — governing statutes for community residences
- N.J.A.C. 10:44A-2.2 — development and maintenance of the procedure manual and continuous quality improvement system
- N.J.A.C. 10:44A-2.6 — new-employee orientation; N.J.A.C. 10:44A-2.9 — records of individuals; N.J.A.C. 10:44A-5.2 — prescription medication
- Danielle's Law (N.J.S.A. 30:6D-5.1 et seq.); Stephen Komninos' Law (N.J.S.A. 30:6D-9.5) — emergency 911 and drug-testing/reporting
- N.J.A.C. 10:46C — Community Care Program waiver; N.J.A.C. 10:46D — contribution to care; N.J.A.C. 10:41 — incident reporting and confidentiality

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